Williams Physical Therapy

93183 Lisa Fort
Baytown, TX
Phone 281-555-9042

Itemized Medical Bill

Patient:	Deborah Lynch
Date of birth:	June 1, 1966
Insurance claim:	C-1052
Date of injury:	November 30, 2024
First visit:	November 30, 2024
Last visit:	March 21, 2025
Number of visits:	18

Services

Service	Units	Charge
99203 New patient evaluation	1	$262.76
98941 Spinal manipulation	3	$363.43
97110 Therapeutic exercise	4	$690.30
97124 Massage therapy	2	$210.28
72040 X-ray, cervical spine	6	$3,476.59
20552 Trigger point injection	2	$826.80
TOTAL		$5,830.16

Recent visits

Date	Service	Charge
February 16, 2025	72040 X-ray, cervical spine	$461.29
February 25, 2025	20552 Trigger point injection	$489.90
March 7, 2025	72040 X-ray, cervical spine	$675.95
March 10, 2025	72040 X-ray, cervical spine	$763.03
March 17, 2025	97110 Therapeutic exercise	$155.81
March 21, 2025	72040 X-ray, cervical spine	$521.62

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Treatment History and Summary

Patient:	Deborah Lynch
Treating provider:	Tommy Griffin, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
November 2024: evaluation	1	$262.76
December 2024: chiropractic adjustment, imaging, massage therapy, pain management, physical therapy	5	$1,411.24
January 2025: chiropractic adjustment, physical therapy	4	$566.69
February 2025: imaging, massage therapy, pain management	4	$1,473.06
March 2025: imaging, physical therapy	4	$2,116.41

Treatment summary

The patient presented following a low-speed collision in a parking lot with complaints of right knee pain, swelling, and reduced range of motion. The mechanism of injury is consistent with the reported knee trauma, and the presenting symptoms are plausible for an impact of this kind. Over the course of 18 visits, care included physical examination, imaging, a structured rehabilitation program, and activity modification. Imaging and clinical examination findings clearly confirm the knee injury. The patient's history inlcudes a small strain to the same region many years ago, which resolved completely without lasting limitation and did not affect the current presentation. Treatment followed a typical clinical course, with visit frequency and intensity decreasing steadily as strength, mobility, and pain improved. Billed services reflect the treatment delivered, with no repeated or duplicate charges. At the most recent evaluation, the patient was progressing well and was discharged to a self-directed home exercise program with follow-up as needed.